PORTFOLIO SAMPLE · MEDICAL FACT-CHECK

Fact-Checking a Patient Article on Type 2 Diabetes

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Medical Reviewer · Oct 2026

1. Brief

A health blog asked for a clinical review of a patient-facing article before publication: check every medical claim against current guidelines, flag errors by severity, and supply corrected wording that keeps the article's plain-language tone.

2. Draft article as submitted (errors highlighted)

Living with Type 2 Diabetes: What You Need to Know

Type 2 diabetes is one of the most common long-term conditions in India. It is caused by eating too much sugar [2], which is why it mostly affects people who love sweets. The good news is that only overweight people need to worry about it [4], so if your weight is normal, your risk is low.

How do you know if your sugar is high? A normal fasting blood sugar is anything under 140 mg/dL [1]. If yours is higher, speak to your doctor.

Diet is the cornerstone of control. People with diabetes should avoid all fruit [3], since fruit is full of natural sugar. Stick to vegetables, whole grains and lean protein.

Finally, your doctor will check your HbA1c every few months. Once it falls below 7%, your diabetes is cured [5] and you can usually stop your medicines.

3. Findings

1 MAJOR **Wrong diagnostic threshold**

CLAIM *"A normal fasting blood sugar is anything under 140 mg/dL."*

FACT Normal fasting plasma glucose is below 100 mg/dL. 100 to 125 mg/dL indicates prediabetes, and 126 mg/dL or higher (confirmed on repeat testing) indicates diabetes. The stated cut-off would falsely reassure readers who already meet diagnostic criteria.

REWRITE A normal fasting blood sugar is below 100 mg/dL. Between 100 and 125 suggests prediabetes, and 126 or above may mean diabetes, so speak to your doctor.

Ref: ADA Standards of Care, Diagnosis and Classification [1]

2 MINOR **Oversimplified cause**

CLAIM *"It is caused by eating too much sugar."*

FACT Type 2 diabetes results from insulin resistance and reduced insulin secretion, driven by excess body fat (especially around the waist), physical inactivity, family history and age. A high intake of sugary drinks raises risk, but sugar alone is not "the cause".

REWRITE It develops when the body stops responding well to insulin. Extra weight around the waist, inactivity, family history and age all raise the risk, and so does a diet high in sugary drinks and refined foods.

Ref: WHO Diabetes fact sheet [2]; ICMR guidelines [3]

3 MAJOR **Unnecessary dietary restriction**

CLAIM *"People with diabetes should avoid all fruit."*

FACT Whole fruit in sensible portions is part of a recommended diabetes diet and provides fibre and micronutrients. Fruit juices and very large portions are what should be limited.

REWRITE You don't need to give up fruit. Choose whole fruit over juice, keep portions moderate, and spread it through the day.

Ref: ADA Standards of Care, Nutrition therapy [1]

4 MAJOR **False reassurance for a high-risk group**

CLAIM *"Only overweight people need to worry about it."*

FACT South Asians develop type 2 diabetes at lower body weights than many other populations, which is why lower BMI thresholds are used for screening Asian adults. People with a normal BMI can still be at high risk, particularly with a family history or central obesity.

REWRITE Being overweight raises your risk, but people of a healthy weight can get it too, especially in South Asian families. Ask your doctor about screening if diabetes runs in your family.

Ref: ADA Standards of Care, screening in Asian adults [1]; ICMR guidelines [3]

5 CRITICAL **"Cured" claim could lead readers to stop treatment**

CLAIM *"Once it falls below 7%, your diabetes is cured and you can usually stop your medicines."*

FACT An HbA1c below 7% is a common treatment target and means diabetes is well controlled, usually because of the treatment. It is not a cure. Remission is defined separately (HbA1c below 6.5% sustained for at least 3 months off glucose-lowering medication) and still needs ongoing monitoring. Patients should never stop medicines without medical advice.

REWRITE For many people the goal is an HbA1c below 7%, which means your diabetes is well controlled. Keep taking your medicines unless your doctor tells you otherwise, and keep up regular check-ups.

Ref: ADA Standards of Care, Glycemic goals [1]; ADA/EASD consensus on remission [4]

4. Reviewer summary

5	1	3	1
claims flagged	critical	major	minor

Tone and structure suit a lay audience, but three claims would mislead readers, one could cause harm if followed, and one is oversimplified. With the rewrites above the article is suitable for publication. Recommended additions: a line on regular foot and eye checks, and a prompt to seek care for symptoms such as excessive thirst, frequent urination or unexplained weight loss.

Verdict: Revise before publishing Apply all five corrections, then re-review.

5. References

1. American Diabetes Association. Standards of Care in Diabetes (current edition): Diagnosis and Classification; Facilitating Positive Health Behaviors; Glycemic Goals.
2. World Health Organization. Diabetes, fact sheet.
3. Indian Council of Medical Research. ICMR Guidelines for Management of Type 2 Diabetes.
4. Riddle MC, et al. Consensus Report: Definition and Interpretation of Remission in Type 2 Diabetes. Diabetes Care. 2021.

Sample work prepared to demonstrate review method. The draft article was written for this exercise and is not from any real publication. Not medical advice for any individual.